

intersex
NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT

INTERSEX FRIENDLY HEALTHCARE TOOLKIT

By Intersex Nigeria (Center for Healthcare Development and Youth Empowerment)

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For permissions or inquiries, please contact:
info@intersexnigeria.org

Contact Information

Intersex Nigeria
(Center for Healthcare Development and Youth Empowerment)

Lagos and Abuja, Nigeria

info@intersexnigeria.org

+2349129057794

Visit our website

www.intersexnigeria.org

Social Media

@Intersex_nigeria_

@IntersexNigeria

@IntersexNigeria

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About Intersex Nigeria

Intersex Nigeria is an intersex led and non -governmental organisation formally established in 2019 and officially registered in Nigeria in 2021. The organisation stands as a symbol of support and empowerment for intersex persons in Nigeria. While Intersex Nigeria is located in Nigeria, the organisation implements across the country and other African countries.

The organisation's overarching goal is to champion the sexual and reproductive health, self determination, bodily autonomy, and fundamental human rights of intersex individuals across Nigeria. We achieve this through a multifaceted approach that includes community building, advocacy, evidence-based research, empowerment initiatives, and the dissemination of educational resources on intersex-related issues to various stakeholders, partners, and the wider community. This involves supporting and empowering intersex individuals and their families through online forums, social activities, and face-to-face meetings; providing information, education, and advice on intersex issues via the internet and information communication and education materials; leading and contributing to policy-making processes that protect and provide comprehensive healthcare services for intersex people; increasing support, awareness, and recognition of intersex rights with commitment from different stakeholders; engaging and partnering with other organisations that share common goals; and raising, applying, and channeling resources to support the standard of living of intersex persons while ensuring their safety and sustained development.

At the core of our vision lies the desire to see a society where intersex persons, from birth, have rights to bodily integrity and autonomy and are free from stigma and discrimination. We envision a future where stigma and discrimination against intersex individuals are eradicated, replaced instead by a culture of inclusivity, visibility, and understanding. Our mission is clear: to create an enabling society that promotes the visibility, inclusion, bodily autonomy, sexual and reproductive health, including mental wellbeing and human rights of intersex persons in Nigeria, through community organising, research, advocacy, empowerment, and public engagements using a feminist lens. We at Intersex Nigeria design all our projects with this mission in mind. Ultimately, our aim is to ensure that intersex individuals in Nigeria, from the moment of birth and throughout their lives, are granted the fundamental rights to bodily integrity and autonomy, liberated from the shackles of stigma and discrimination. Through our tireless efforts, we strive to pave the way towards a future where every intersex person can thrive, unimpeded by societal prejudices.

Acknowledgement

Intersex Nigeria expresses their deepest gratitude to all those who contributed to the development of this "intersex-friendly healthcare toolkit." This initiative would not have been possible without the unwavering support, dedication, and insights of numerous persons and organisations.

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With gratitude,
Intersex Nigeria Team

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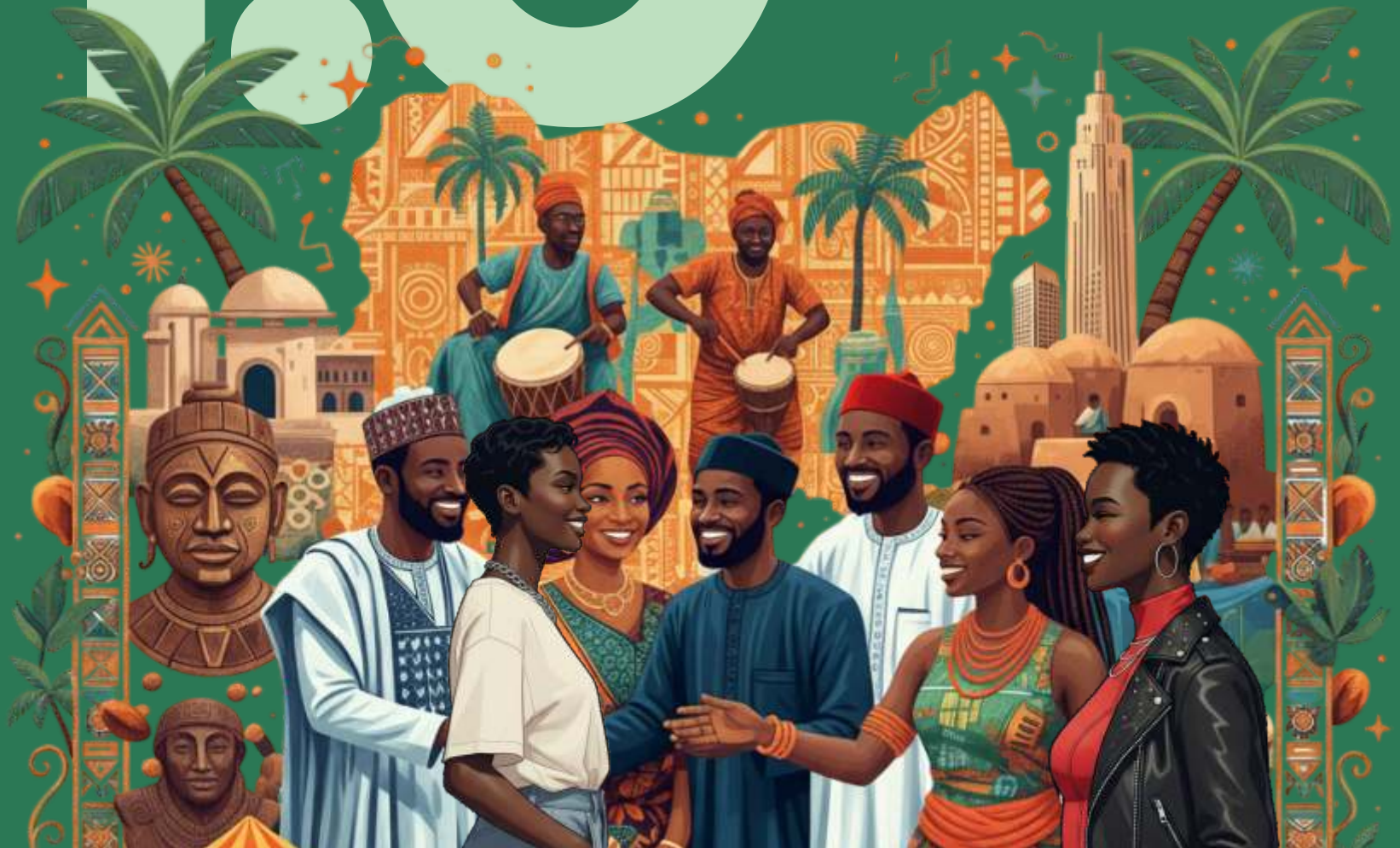
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1.0 Introduction



1.0 Introduction

1.1 Purpose of the Toolkit

The goal of the Intersex-Friendly Healthcare Toolkit is to give healthcare professionals, policymakers, and civil society organisations (CSOs) the information, strategies, and moral guidelines they need to provide intersex people with services and interventions that are welcoming, respectful, and sensitive. Its key objectives include:

1. **Education and Awareness:** To raise awareness among healthcare providers about intersex variations and challenge outdated medical practices that prioritise the normalization of intersex bodies over patient autonomy.

2. **Ethical Medical Interventions:** To promote best practices for patient-centred care, emphasising informed consent and bodily autonomy before any medical intervention, particularly non-emergency surgeries on intersex infants and children.

3. **Cultural Competence:** To foster an understanding of cultural dynamics, especially in regions like Nigeria, and provide guidance for navigating sensitive conversations with patients and families.

4. **Advocacy and Empowerment:** To equip intersex individuals and their families with the tools for self-advocacy, ensuring they can navigate healthcare systems with confidence and dignity.

5. **Policy Development:** To provide frameworks for developing intersex-inclusive healthcare policies, supporting healthcare institutions in creating a more welcoming environment for intersex patients and their families.

1.2 Importance of Intersex Friendly Healthcare in Nigeria

In Nigeria, intersex individuals face significant challenges, including social stigma, medicalisation, and lack of awareness. Societal pressure to conform to binary gender norms drives many intersex people to undergo unnecessary medical interventions without consent from birth. This lack of understanding and respect for intersex autonomy leads to long-term physical and psychological harm. Intersex-friendly healthcare is important in Nigeria for the following reasons:

1. **Reduces stigma:** Due to their variations in sex characteristics, intersex people often face marginalisation. Creating healthcare environments that are inclusive and sensitive to these differences can help reduce societal stigma.

2. **Promotes Informed Consent:** Without their consent, intersex individuals have historically endured harmful, irreversible surgeries. Prioritising informed consent ensures that intersex people control their decisions regarding their bodies.

3. **Addresses Cultural Barriers:** In Nigeria's culturally diverse society, traditional and religious beliefs about gender often lead to misunderstandings of intersex individuals. Intersex-friendly healthcare can bridge these gaps by educating communities and providing respectful care that aligns with both cultural values and human rights.

4. **Improves Mental Health:** Many intersex individuals experience trauma due to discrimination and medical interventions. Providing access to supportive healthcare acknowledges their identities and needs and can improve their mental health outcomes.

1.3 Target Audience

The toolkit is designed for:

- **Healthcare Providers:** doctors, nurses, and healthcare workers who interact directly with intersex patients. It equips them with the knowledge and strategies needed to provide non-discriminatory and ethical care.

- **Policymakers:** government officials and health policymakers who can influence healthcare systems and legal frameworks to protect intersex individuals.

- **Community Leaders and NGOs:** organisations working in healthcare advocacy and human rights, who can use the toolkit to educate communities and push for policy reforms.

- **Families of Intersex Individuals:** Parents, guardians, and families can use this toolkit to better understand intersex variations and advocate for the rights and health of their loved ones.

1.4 How the Toolkit Can Be Used

1. **Educational Resource:** The toolkit serves as an educational guide for training healthcare professionals, ensuring they are equipped to handle intersex-related healthcare issues with sensitivity and respect.

2. **Policy Development:** It provides frameworks for healthcare institutions and government bodies to develop and implement intersex-inclusive policies that safeguard patient rights.

3. **Advocacy Tool:** NGOs and advocacy groups can use the toolkit to campaign for better healthcare services, legal protections, and societal understanding of intersex issues.

4. **Support for Families:** The toolkit offers resources for families to understand intersex variations and how to navigate medical systems to ensure their intersex family members receive appropriate care.



2.0

Understanding Intersex



2.0 Understanding Intersex

2.1 Definition of Intersex Variations

Intersex variations refer to a wide range of natural bodily differences in sex characteristics that do not fit typical binary definitions of male or female. These variations may be evident at birth, during puberty, or sometimes may never be identified unless a person undergoes medical testing. They can involve differences in chromosomes, gonads (testes or ovaries), sex hormones, or genitalia.

Over 46 different intersex variations exist. These conditions are characterised by differences in chromosomal patterns, external genitalia, and so much more. Intersex variations can affect people differently, so there is no singular experience that all intersex people have.

2.2 Diversity of Intersex People

It is important not to lump all intersex people into one new collective category, such as a "third sex," perhaps running in parallel to female and male. Such a classification would be incorrect due to the enormous diversity among intersex people and the fact that many intersex individuals do identify as women or men, while others identify as both or neither. In effect, intersex is an umbrella term including people with "variations in sex characteristics" rather than a type per se. This diversity is not unique to intersex people, as unsurprisingly, a range of variations in sexual anatomy is also found in women and men that meet the medical norms of their respective categories.

The term "hermaphrodite" was widely used by medical practitioners during the 18th and 19th centuries before "intersex" was coined as a scientific and medical term in the early 20th century. Before the current medical classification of the disorder of sex development (DSD) was developed, variations in intersex sex characteristics were classified under different categories, the most common being: congenital adrenal hyperplasia (CAH), androgen insensitivity syndrome (AIS), gonadal dysgenesis, hypospadias, and unusual chromosome compositions such as XXY (Klinefelter Syndrome) or XO (Turner Syndrome). The so-called "true hermaphrodites" referred to those who combined their ovaries and testes.

Importantly, variations in sex characteristics are different from sexual orientation and gender identity, even though the three layers interact in the formation of a person's personality. The Office of the United Nations High Commissioner for Human Rights' (OHCHR) Free & Equal campaign points out that "intersex people experience the same range of sexual orientations and gender identities as non-intersex people." In this vein, reference to intersex people as "intersexuals" is wrong since intersex sex characteristics are unrelated to sexual orientation. Similarly, reference to "intersex identity" is also incorrect, as intersex is not necessarily a matter of identity or self-perception but mostly refers to innate/physical aspects of the body.

Most discrimination against intersex persons is motivated by homophobia and/or transphobia, which connects their experiences

to those of other marginalised populations or other sexual minority groups. Examples include: "Parental choice against intersex may conceal biases against same-sex attractiveness and gender nonconformity." Society's sex and gender roles often lead to human rights violations against sexual minorities. This is especially true when heteronormativity and conformity to the sex assigned at birth are combined with prejudiced ideas that value heterosexuality. Stereotypical human rights reinforce these dichotomies by allowing women and men to have different appearances and roles, and by imposing legal and/or social distance between them.

Other marginalised groups, such as persons with disabilities and those who have experienced genital mutilation or cutting, also share some human rights concerns with intersex people. Intersex people may find the well-established legal framework protecting people with disabilities, which is far-reaching in several countries, useful in protecting their rights or serving as a reference for the development of intersex-specific legislation.

Furthermore, it's important to acknowledge that the prevalence of intersex individuals may not be as uncommon as commonly perceived. While an expert at a health centre is likely to quote a figure between 1 in 1500 and 1 in 2000, based on an estimate on the number of newborns diagnosed as intersex, many people are born with subtler forms of sex variations that are not immediately detectable. This latter group, however, still does not meet medical standards of female and male and may be subjected to medical interventions at a later stage. In Anne Fausto-Sterling research through medical literature for frequency estimates, she concluded that around 1.7% of human births are intersex.[1]

[1] *https://rm.coe.int/16806da5d4*
[2] *https://interactadvocates.org/intersex-definitions/*

2.3 Common Intersex Variations and their Characteristics

According to the intersex advocacy organisation interACT, there are over 46 defined terms for different intersex variations.[2] Some examples of these variations include:

46, XX Salt-Wasting Congenital Adrenal Hyperplasia (SWCAH)

A baby with XX chromosomes and CAH usually has uterus, ovaries, and genitalia that may appear visibly different. This may include having a large clitoris, a short vagina, and/or larger or different labia. Babies with other types of CAH may not have a genital difference.

While genital differences on their own are almost never life-threatening, infants with CAH also have serious medical needs due to how their bodies produce stress hormones. 46, XX salt-wasting CAH (SWCAH) happens when there is a genetic mutation in enzymes of the adrenal gland, which sit on top of the kidneys. These enzymes are chemicals needed by the adrenal gland to make cholesterol into three important hormones that help to regulate the body's functions: cortisol, aldosterone, and androgen. If one of the enzymes needed to make cortisol and/or aldosterone is not working properly, the adrenal glands fail to work in a balanced way. They make too little cortisol and/or aldosterone and more androgen than usual. When there is not enough cortisol or aldosterone, babies with SW CAH may become very sick. They can become dehydrated and lose blood pressure if not treated urgently.

Androgen Insensitivity Syndrome (AIS)

AIS happens when babies are born with testes and XY chromosomes, but their bodies are immune or unable to respond to androgens, a category of hormones including testosterone. Since a

penis and scrotum develop under the influence of androgens, a baby with AIS may develop with genital differences, depending on their level of immunity to androgens.

Babies with *complete* AIS (CAIS) have *no* response to androgens and usually develop a vulva and typically sized clitoris, while babies with *partial* AIS (PAIS) may have genital differences, with external genitalia that appear on a spectrum. During development, the testes also make a hormone (Mullerian inhibiting substance, or MIS) that prevents formation of the uterus, fallopian tubes, and a small part of the upper vagina. Children with testes and less sensitivity, or ability to respond to androgen, may go through a feminising puberty on their own. Because testes do make some estrogen, and the body can turn androgen into estrogen, these children may have traits like breast development from their own hormones at puberty and often supplement that with synthetic hormones to develop fully. Surgical removal of the testes (gonadectomy) may be recommended in PAIS because there may be a risk of cancer developing in the testes. The risk of cancer development before puberty in CAIS is very low, and it is an accepted medical practice to leave CAIS testes in place for a more natural puberty. Because androgen also influences sperm formation, some adult men do not discover that they have minimal PAIS until they see a doctor for fertility tests.

46, XY complete gonadal dysgenesis (Swyer syndrome)

Swyer syndrome occurs when a baby is born with XY chromosomes, but the testes do not develop. During prenatal development, these babies develop a vulva and a small uterus. The underdeveloped "would-be testes" become fibrous tissue called "streaks," which are neither testis nor ovary. These children need to take hormones in order to start puberty. Because there is an increased risk of cancer developing in streak gonads, removal is commonly recommended.

46, XY partial gonadal dysgenesis

Partial gonadal dysgenesis in a baby with XY chromosomes causes development of testes that do not function at the same level as typical testes. Sometimes the testes disappear or regress. Babies' genitals can vary in appearance depending on how much the testes function. Tumours occur about 20–30% of the time.

5 alpha reductase-3 deficiency (5 ARD deficiency) and 17beta-hydroxysteroid dehydrogenase-3 deficiency (17 BHSD deficiency)

In 5 alpha reductase-3 deficiency (5 ARD deficiency) and 17 beta-hydroxysteroid dehydrogenase-3 deficiency (17 BHSD deficiency), genetic mutations in XY babies with testes result in atypical levels of sex hormones. All hormones are made from cholesterol by enzymes, and these mutations change the function of enzymes needed to make androgens. Children with 5 ARD deficiency and 17 BHSD deficiency have XY chromosomes and testes, but their testes make a version of androgen that is weaker than usual. Babies may have a vulva or vary along the spectrum of genital difference. At puberty, when levels of androgen increase dramatically, the large amount of weaker androgens can cause changes such as deepening of the voice, growth of facial hair, and muscle development. If a child does not want these changes, puberty can be blocked by medication. There are reports of fertility in adults with 5 ARD, but not 17 BHSD deficiency.

Ovotesticular DSD
In typical sex development, the fetus develops two tiny organs called "proto-gonads." Usually these proto-gonads will become either testes or ovaries. Sometimes an ovotestis develops instead, which contains some ovary-type cells and some testis-type cells. Although a person may be born with two ovotestes, it is much more common for a person to be born with a typical ovary or testis on one side and an ovotestis on the other. People with ovotestes may have a typical genital appearance at birth, with either a vulva and labia or penis and scrotum, and some look more in-between. Because there may be many different chromosomal and internal anatomical combinations, children may need other tests to evaluate their unique situations. The testicular part of ovotestes may have an increased cancer risk.

Mayer-Rokitansky-Küster-Hauser (MRKH)
Children with 46, XX chromosomes and ovaries may have atypical development of internal structures such as the vagina, the uterus, and the Fallopian tubes. MRKH sometimes also involves differences in development of the skeleton, internal ears, and in rarer cases, the heart, fingers and toes.

Hypospadias and Epispadias
Hypospadias happens when the urinary opening (urethra) is located below the usual position on the tip of the penis. It is one of the most common and visible genital variations. Hypospadias is the most common birth difference in children with XY chromosomes, seen in 1 out of every 125 to 300 live births. The cause of hypospadias is not known. The location of the opening of the urethra can vary from just below the tip of the penis down to just in front of the rectum. Bending of the penis, or chordee, is often seen with hypospadias, although they can occur separately.

Epispadias is similar to hypospadias because the urinary opening is not in its usual position. In epispadias, the urinary opening is located on the upper surface of the penis or in the middle of the clitoris. It is much less common than hypospadias. Epispadias usually happens in conditions called exstrophy, where the bladder, abdominal wall, and pelvis are not closed in the front as usual, but epispadias can also occur by itself.

X0 Turner Syndrome (TS)
Turner Syndrome happens when a person has one complete X chromosome, and their second X chromosome is absent or smaller than usual. This happens in about 1 in 2000 live births. Turner syndrome is sometimes found prenatally with amniocentesis. In TS, the level of hormones needed to start puberty is unusually low because of a difference in development of the ovaries, which are called "streak gonads." TS can also be discovered in adolescence when puberty does not happen as expected. Many people with TS are petite, often under 5 feet tall. Other organs in the body, such as the heart, may also develop atypically.

Some people only have one X chromosome in some of their cells. This is called "mosaic Turner Syndrome." The other cells can have XX or XY chromosomes. Physical difference in mosaic TS will depend on what proportion of the cells and tissues have typical XX or XY chromosomes.

XXY Klinefelter syndrome
Klinefelter syndrome (or XXY syndrome) is a genetic condition in which a person has an extra copy of the X chromosome. Klinefelter syndrome is not inherited, but rather occurs only as a result of a random genetic error after conception. People born with Klinefelter syndrome may have low testosterone and reduced muscle mass, facial hair, and body hair. Most people with this condition produce

little or no sperm. Some may choose testosterone replacement and fertility treatment.[3]

2.4 Prevalence of Intersex Traits and Diversity within Intersex Population

Intersex individuals are born with variations in sex characteristics that do not fit typical definitions of male or female bodies. The diversity within the intersex population is broad; it encompasses a wide range of chromosomal, hormonal, and anatomical variations. It is important to note that intersex is not a single variation but rather an umbrella term for a number of differences in sex development (DSDs). In some cases, intersex differences are apparent at birth, while others may not be identified until puberty or even later in life, and some may never find out throughout their lifetime. Intersex variations are incredibly diverse. Some examples of chromosomal, hormonal, and anatomical variations are;

Chromosomal Variations: Individuals may have atypical chromosomal patterns such as:

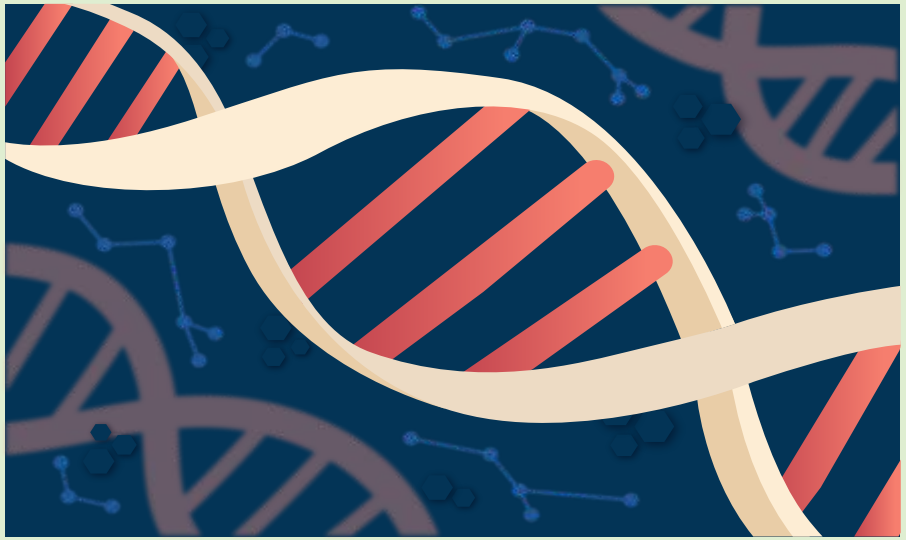
1. Klinefelter syndrome (XXY), where individuals have an extra X chromosome.
2. Androgen Insensitivity Syndrome (AIS), where an individual with XY chromosomes has body tissues that do not respond to testosterone.
3. Turner syndrome (XO), where individuals may have only one X chromosome or a partial second chromosome.

[3] *https://interactadvocates.org/intersex-definitions/*

Hormonal Variations: Some individuals may have typical male or female chromosomes but atypical hormone production or reception, leading to differences in development. For example:

1. Congenital Adrenal Hyperplasia (CAH), which affects cortisol and androgen production, often results in ambiguous genitalia in individuals with XX chromosomes.
2. 5-Alpha-Reductase Deficiency, where individuals with XY chromosomes have a form of testosterone that doesn't convert properly, often leading to ambiguous genitalia or female presentation at birth.

Anatomical Variations: Some intersex traits are visible at birth in the form of ambiguous genitalia, while others may involve internal reproductive organs that don't match external genitalia. This can include individuals with ovotestes (both ovarian and testicular tissue) or atypical gonads.



2.5 Other FAQs About Intersex Variations

Intersex variations can affect intersex people differently, and there's no "one" intersex experience. Whether you have recently learned that you are intersex or you are curious to learn more about what it means to be intersex, here are a few more things you should know.

Which is the rarest form of intersex?
According to Özdemir Meltem and Co.[4] the rarest form of intersex variation is ovotesticular DSD, also called ovotestes. Ovotesticular DSD was previously known as "true hermaphroditism," but intersex individuals choose to no longer use this term, as it has become stigmatising.

Can you be intersex and not know it?
Yes. Intersex traits can vary from person to person, and not every intersex individual has outwardly visible changes in their sex characteristics. Some people may never know that they are intersex, while others become aware later in life through personal experiences or medical testing.

Do intersex people have periods?
Several factors can affect whether intersex persons can experience periods irrespective of their gender identity. Intersex variations affect different sex characteristics, and at the end of the day, whether or not an intersex person has periods will depend on how their intersex traits affect their body.

What is the difference between intersex and hermaphrodite?
"Hermaphrodite" is an outdated and misleading term that medical professionals previously used to describe people with intersex variations. Many intersex people consider the term, hermaphrodite a slur and prefer to use "intersex" to describe themselves and their experiences.

Is being intersex the same as being transgender?
No. Although sex and gender are related, they are not the same thing. However, some intersex people may also be transgender, depending on how their feelings align with the gender they were raised as. It is also possible that an intersex person's concept of their gender may change if their body goes through changes they weren't expecting during puberty.

Does Polycystic Ovary Syndrome (PCOS) qualify as an intersex variation?
The answer to this question is still highly contested among medical experts and the intersex community. Currently, PCOS is not medically considered an intersex variation. But many people with PCOS do experience intersex-like changes to their hormonal balance, genitals, secondary sex characteristics, and more.

[4] *https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6737443/*

2.6 Psychological, Medical, Social And Cultural Considerations

The experiences of intersex individuals are shaped not only by their biological traits but also by deep rooted beliefs and aspects such as psychological, social, cultural, and medical attitudes. Historically, many intersex individuals have been subjected to non-consensual medical interventions, including surgeries and hormone treatments aimed at making their bodies conform to binary gender norms. These practices have been widely condemned by human rights organisations including the United Nations as violation of bodily autonomy.

Many intersex individuals in Nigeria and Africa at large experience severe stigma and struggles with self-esteem due to the limited understanding around intersex variations. Intersex children and persons are constantly being perceived as different or taboo because of their bodies, and this negatively impacts their psychological wellbeing and self-esteem. In Nigeria, many intersex variations are seen through a lens of superstition or religious beliefs that view intersex people as cursed or abnormal. Relationships are often complicated by societal expectations and the fear of disclosure. Most intersex people avoid forming and maintaining close relationships because of the fear of rejection, judgements, or discrimination once their intersex status is known. In Nigeria, most families of intersex people hide the existence of their intersex child/sibling to avoid societal judgements. The pressure to maintain secrecy around intersex children is high, and this often prevents access to necessary medical and psychological care.

> *There is a growing intersex rights movement in Nigeria, with Intersex Nigeria championing the visibility and awareness since 2019 as well as in other African countries, with focus on promoting intersex advocacy, bodily autonomy, informed consent, legal recognition, and the right to self-identification. Intersex people, like any other group, have a diverse range of gender identities and expressions, and many activists also advocate for greater awareness of the intersex spectrum.*
>
> *Successful advocacy for intersex rights in Nigeria must be culturally sensitive and context-specific, addressing the unique socio-cultural dynamics in each culture and geopolitical zone. The intersex population is diverse in terms of biology, identity, and experience. With continued advocacy, Nigerian society is slowly recognising the need for inclusive approaches that respect intersex individuals' rights to bodily integrity and self-determination.*

2.7 Dispelling Myths And Misconceptions

Myth: Being intersex is extremely rare.

Fact: The scientific consensus is that between 1.5 and 2% of the world population is intersex

Myth: Intersex is a condition that needs 'correcting'.

Fact: Intersex people, including children, are rarely harmed by their natural divergences, unnecessary 'treatment' can cause lifelong damage to an intersex individual.

Myth: Intersex people have 'both' genitals or are 'both' sexes.

Fact: The truth is, it is complicated. Sex is a series of traits and spectrums, and there are many ways human bodies can develop. Having both a penis and vagina is not one of them.

Myth: All intersex people have ambiguous genitalia at birth and need surgeries.

Fact: Not all intersex people have ambiguous genitalia at birth, not all intersex experiences involve surgery, and not all intersex medical harm is surgical. Many intersex people find out they are intersex by way of hormonal differences during puberty or adulthood.

Myth: Intersex people are transgender.

Fact: The word "transgender" or trans is an umbrella term for people whose gender identity is different from the sex they were assigned at birth. The word "intersex" relates to innate/physical sex characteristics and not an internal sense of identity.

Myth: Intersex people all have non-binary gender.

Fact: Intersex people can have any gender.

Myth: Intersex people can't have sex, get pregnant, or have children.

Fact: Many intersex variations are typical internal reproductive anatomy.
Everyone is different. There are intersex people who can carry children, and intersex people who can produce sperm.
There are some who can not. There are also intersex people who find out they have both ovarian and testicular tissue after giving birth.

Myth: No one is speaking out for intersex people

Fact: Although there are still a lot of misconceptions around the meaning of intersex, there are lots of amazing intersex activists around the world who are fighting to raise awareness and protect human rights.



2.8 Importance of Using Respectful And Appropriate Language

Language has the power to either affirm the humanity of intersex people or perpetuate stigma, discrimination, and trauma. The way healthcare providers communicate with patients can have a profound impact on the quality of care and the overall experience of their patients. Same thing for intersex individuals; the importance of using respectful, inclusive, and appropriate language in healthcare settings cannot be overemphasised. In Nigeria, where traditional gender roles are strictly enforced and awareness about intersex variations is low, healthcare settings often become places where intersex individuals encounter discrimination, misunderstanding, and sometimes trauma.

The use of sensitive, respectful, and informed language in intersex healthcare helps with combating medical stigmatisation, avoids pathologisation, builds trust between healthcare providers and intersex patients, and promotes bodily autonomy and informed consent. In Nigeria, healthcare practitioners lack comprehensive education on intersex variations. Due to this gap, the use of harmful and outdated language such as 'hermaphrodite' is common and it is derogatory, medically inaccurate, and offensive to intersex people and their human rights. Using stigmatising terms that dehumanise intersex individuals reinforces societal myths that intersex bodies are unnatural or need to be "fixed." Respectful and inclusive language, such as referring to intersex variations as simply "sex variations," helps to combat medical stigmatisation and presents intersex traits as part of the natural diversity of the human body.

A recent study by Intersex Nigeria showed that within Nigerian healthcare, there is often pressure on families and intersex individuals to undergo medical interventions aimed at correcting their intersex traits; this happens most times without the informed consent of the patient. These medical interventions are frequently based on societal discomfort with bodies that do not conform to binary gender norms. For most intersex people, healthcare settings are sites of significant trauma due to past experiences of non-consensual procedures, misdiagnosis, or outright discrimination. The use of respectful and affirming language is the first step in rebuilding the broken trust between intersex patients and healthcare providers and shifting the healthcare approach from coercive interventions to one that empowers intersex individuals to make their own informed decisions about their bodies.

Disrespectful language and misgendering can make it difficult for people to participate fully in our communities and services. We often misgender people without even noticing that we have done so, and we often do not realise how our words can exclude people. Those frequently misgendered by others remain silent, despite the significant distress misgendering can cause them. Some people take longer than others to learn how to use inclusive language. We can politely and consistently model respectful and inclusive language or clarify a person's preferred language with others. By speaking up and modelling inclusive practice, we can make sure that all of us have a say in our communities and services.[5]

When you misgender someone, apologise and start using respectful, gender-neutral language. In that moment, prolonged discussion about misgendering is likely to make the person who was misgendered feel worse. After leaving, you can think about how to be more inclusive.

[5] *https://www.parliament.nsw.gov.au/lcdocs/other/12075/Inclusive%20language%20guide%20-% 20Respecting%20people%20of%20intersex,%20trans%20and%20gender%20diverse%20experience.pdf*

2.9 Evolving Language and Concepts in Intersex Care.

Intersex care has for long been framed within the context of medicalisation, often driven by the need to correct bodies that deviate from the binary gender norms. Over the past few decades, the language and concepts surrounding intersex care have evolved and now reflect a growing understanding of human diversity and a shift towards patient-centred, rights-based approaches. This evolution has been largely driven by intersex advocates, human rights organisations, and progressive healthcare professionals who recognise the need to move beyond outdated practices rooted in stigma and non-consensual medical interventions, of which the latter is still a huge struggle within the Nigerian context.

As the global and African intersex movement continues to grow and gain visibility, healthcare professionals, policymakers, and communities are increasingly embracing more inclusive, respectful language and frameworks that prioritise the wellbeing and autonomy of intersex individuals. This shift is not only necessary for improving the quality of care for intersex patients but also for fostering a broader societal acceptance of bodily diversity. [6]

The Shift from Pathology to Diversity

One of the most significant shifts in intersex care has been the move away from pathologising intersex traits toward understanding them as part of natural human variation. Historically, intersex traits were viewed as medical "disorders" that needed to be corrected, often through invasive surgeries or hormone treatments that were imposed on children without their consent. These interventions



were justified by a belief that atypical bodies needed to be made to fit binary male or female norms to allow individuals to function in society.

Old Terminology: Pathology and Medicalisation

- **"Disorders of Sex Development (DSD)":** This term was introduced in the early 2000s as a way to replace the outdated and offensive term "hermaphrodite." While "DSD" was an improvement, it still framed intersex traits as disorders, contributing to a medicalised view of intersex bodies as problems that needed to be fixed.

[6] *https://oiieurope.org/wp-content/uploads/2019/05/Protecting_intersex_in_Europe_toolkit.pdf*

- **"Abnormal genitalia" or "ambiguous genitalia":** These terms were commonly used in medical settings to describe intersex traits, reinforcing the notion that intersex bodies were deviations from the norm and needed to be surgically altered to conform to societal expectations of male or female anatomy.

Evolving Terminology: Diversity and Natural Variation
As the understanding of intersex bodies and experiences has grown, new language has emerged that reflects respect for bodily diversity and moves away from pathologising intersex traits:

- **"Differences in Sex Development (DSD)":** While some continue to use this term, it has been reframed by advocates and healthcare professionals to emphasise diversity rather than disorder. For example, some prefer the term "variations in sex development," which avoids the negative connotations of "disorder."

- **"Intersex traits" or "sex variations":** These terms emphasise that intersex characteristics are natural variations in human development, rather than anomalies that require correction. By focusing on the diversity of bodies rather than on medicalised categories, these terms promote a more inclusive and accepting view of intersex people.

- **"Atypical" or "diverse" genitalia:** These terms replace older, more stigmatising language by recognising that bodies naturally come in many forms, all of which are valid.

This linguistic shift from "disorders" to "variations" is crucial in changing how intersex people are perceived both in medical settings and in broader society. By emphasising diversity and normalising intersex traits, healthcare providers can help reduce the stigma that has long been associated with intersex bodies.

From Forced Interventions to Bodily Autonomy
In the past, intersex care was characterised by early and often irreversible medical interventions, particularly surgeries aimed at making intersex bodies conform to binary male or female norms. These surgeries were typically performed on infants or young children, long before they were able to consent or understand the implications of such procedures. The underlying assumption was that these surgeries were necessary to ensure that intersex children could "fit in" socially and psychologically.

Old Concepts: Urgency and Correction

- **“Normalisation surgeries":** These procedures were framed as necessary to help intersex individuals lead "normal" lives. The language of "normalisation" implied that intersex traits were inherently problematic and that medical intervention was required to make a person "normal.”

- **"Gender assignment surgeries":** These surgeries were performed to assign a gender to intersex infants, often based on the appearance of external genitalia. The assumption was that gender needed to be assigned as early as possible to avoid confusion and that surgery was the best way to achieve this.

Evolving Concepts: Autonomy and Informed Consent
Advocacy from intersex individuals and organisations has led to a rethinking of these practices, with a new focus on respecting bodily autonomy and ensuring that any medical interventions are based on informed consent.

- **"Bodily autonomy":** This concept is central to the evolving understanding of intersex care. Bodily autonomy emphasises that intersex individuals have the right to make decisions about their own bodies, including whether or not to undergo medical procedures. Rather than rushing to perform surgeries on infants, healthcare providers are increasingly recognising the importance of allowing individuals to grow up and make informed choices about their bodies.

- **"Patient-centered care":** In contrast to older models of intersex care, which prioritised conformity to societal norms, patient-centered care focuses on the needs, preferences, and well-being of the individual. This approach involves working with intersex patients (and their families) to provide information, support, and options without pressuring them into making decisions they are not ready for.

- **"Informed consent":** A cornerstone of modern intersex care, informed consent ensures that intersex individuals (or their families, when appropriate) have access to complete and accurate information about their medical options and the potential risks and benefits of different interventions. This concept prioritises the patient's right to understand their variation and participate in decisions about their care.



3.0

Healthcare Needs of Intersex Individuals



3.0 Healthcare Needs of Intersex Individuals

3.1 General Healthcare Considerations

3.1.1 Common Health Issues And Needs of Intersex Individuals

Intersex individuals can experience a range of health issues and needs that are influenced by their unique biological characteristics, societal pressures, and the medical interventions they may have been subjected to during childhood. These needs go beyond the physical and extend into psychological, social, and legal domains. Understanding and addressing these issues in a comprehensive, patient-centred way is crucial for improving health outcomes and overall well-being for intersex people.

Here are some common health issues and needs of intersex individuals:



1. Physical Health Issues

Intersex people may face specific physical health challenges related to their variations in sex development (VSD), that will include

- **Hormonal imbalance:** this has to do with issues of hormonal regulation; where some intersex people have conditions that result in atypical hormone production and hormone replacement therapy (HRT) and where individuals have underdeveloped or removed gonads, they may require lifelong hormone replacement therapy to maintain hormonal balance. Managing hormone levels, especially during puberty, is important to support healthy development in intersex people.

- **Fertility Concerns:** Many intersex variations can affect fertility. Some intersex individuals might not have fully functional reproductive organs or may have undergone procedures like gonad removal that affect their fertility. There is a lack of information on fertility in Nigeria, a worse situation for intersex people, especially those who were subjected to surgeries or medical interventions at a young age. For those who wish to conceive, better communication and access to fertility preservation options and assisted reproduction services are important aspects of care.

- **Increased Risk of Cancer:** Some intersex variations, such as complete androgen insensitivity syndrome (CAIS) or conditions where undescended testes are present, may increase the risk of gonadal tumors. As a result of this, medical protocols should prioritise cancer surveillance and management plans that respect bodily autonomy. Intersex individuals should be fully informed of their options, including monitoring versus prophylactic removal of gonads

- **Complications from Early Surgeries:** Genital and urogenital health is what many intersex individuals who were subjected to genital surgeries in infancy or early childhood, such as clitoral reduction, vaginoplasty, or gonadectomy, often face. These surgeries, in most cases, lead to complications later in life such as scarring or chronic pain in the genital area, urological issues such as difficulty urinating or urinary tract infections (UTIs), and reduced sexual function or pleasure. Long-Term Surgical Monitoring for Intersex Individuals who have undergone surgeries may need ongoing treatment for these complications.

2. Psychological and Mental Health Issues

- **Trauma from Medicalisation:** Many intersex people have endured psychological trauma resulting from early medical interventions, which are often performed without their consent. These surgeries, aimed at "normalising" their bodies to fit societal gender norms, instead lead to long-term emotional distress, including feelings of betrayal and violation. These early medical interventions affect their body image and self-perception, especially if the person does not identify with the gender assigned to them through these surgeries. Many intersex people experience gender dysphoria related to medical procedures or hormonal treatments.

- **Depression and Anxiety:** Due to the stigma surrounding intersex bodies globally and in Nigeria, many intersex people experience anxiety, depression, and feelings of isolation. The lack of supportive, affirming environments (in families, healthcare, and society) makes these mental health challenges even worse. There is a need for psychosocial support. Intersex individuals should have access to benefit from therapy and mental health counselling that is trauma-informed and gender-affirming. Counselling that specifically addresses the psychological impact of medicalisation, body image issues, and social stigma is critical for long-term mental health.





- **Identity and Self-Exploration:** Many intersex individuals struggle with their gender identity and experience gender dysphoria particularly if they were assigned a gender at birth that does not align with their own sense of self. This in most cases leads to internal conflict and identity confusion. Access to gender-affirming counselling is important; it supports individuals to explore their identity in a safe and supportive way without pressure to conform to binary gender expectations. Intersex people should also have access to gender-affirming medical care if they wish to transition or modify their gender expression.

3. Social and Legal Health Needs

- **Stigma and Social Isolation:** In many cultures, including Nigeria and other parts of Africa, intersex individuals face significant stigma due to traditional views of gender and sex. Intersex traits are often misunderstood, leading to ostracism, discrimination, and rejection by families and communities. When we begin to establish safe spaces, peer support groups, and advocacy networks where intersex individuals can connect with others who share similar experiences, they will begin building resilience and reducing social isolation.

3.1.2 Age-specific Healthcare Needs

Intersex people experience unique healthcare needs that vary significantly throughout different stages of their lives. These needs are not only related to their physical development as we mentioned above but also involve psychological, social, and emotional aspects that require specific attention at different ages. Understanding these needs within an age-specific context is critical to providing comprehensive, patient-centred care that respects the autonomy, well-being, and identity of intersex individuals, especially for us here in Nigeria.

Below is a broad outline of age-specific healthcare needs for intersex people:

1. **Pediatric Healthcare Needs (Infancy to Early Childhood)**

Key Considerations:

- **Diagnosis and Monitoring:** Many intersex variations are identified at birth or during infancy, often through physical examinations, genetic testing, or hormonal assessments. In the pediatric phase, it is important for healthcare professionals to accurately diagnose the intersex variation while avoiding unnecessary and invasive procedures that may cause harm or distress to the intersex baby.

- **Non-Emergency Medical Interventions:** One of the most significant issues in intersex healthcare for infants and young children is the historical tendency to perform irreversible surgical procedures aimed at "normalising" genital appearance, also called intersex genital mutilation (IGM), just like FGM. Human rights and medical ethics emphasise the importance of avoiding non-emergency interventions on intersex children until the child can provide fully informed consent.

- **Parental Guidance and Support:** Paediatric care often focuses on guiding and educating parents about the child's intersex variation. Healthcare providers must offer transparent and non-stigmatising information to help parents understand the importance of rejecting non-urgent surgeries. Families of intersex children should be connected with counselling services and intersex-led and focused organisations like Intersex Nigeria to reduce anxiety and provide support for the child.

2. **Healthcare Needs in Adolescence**

Key Considerations:

- **Puberty and Hormonal Management:** During adolescence, some intersex individuals experience puberty differently depending on their variation. Hormonal treatments may be offered to assist with the development of secondary sex characteristics, but these should be presented as options rather than imperatives. It is important that adolescents be given the time and information they require to explore their gender identity without being forced into hormonal interventions.

- **Informed Consent and Decision-Making:** Adolescence is a time when most intersex individuals become more aware of their bodies and gender identities. Medical decisions should be made with the full involvement of the adolescent, ensuring they have the autonomy to make informed choices about their healthcare. Discussions about potential surgeries, hormonal treatments, or other interventions should be approached with caution, centred on the adolescent's right to self-determination.

- **Psychosocial Support:** Adolescents often face heightened social and emotional challenges, particularly regarding body image, identity, and peer relationships. Intersex adolescents may experience gender dysphoria, feelings of isolation, confusion, or stigma due to their unique physical traits or related to medical procedures or hormonal treatments. Comprehensive mental health support, including access to counselling, peer support, and intersex advocacy groups, is helpful with promoting positive mental health and self-esteem.

- **Sexual Health Education:** Adolescents should receive accurate, age-appropriate sexual health education that addresses their specific needs. This includes discussions about fertility, sexual orientation, sexual function, and healthy relationships in a non-judgemental and inclusive manner.

3. **Healthcare Needs in Adulthood**

Key Considerations:

- **Physical Health Maintenance:** Adults with intersex variations may require ongoing medical care related to their physical health, including hormone management, reproductive health, and the management of any surgical outcomes from earlier interventions. It is important that healthcare providers are knowledgeable about intersex variations to offer appropriate care, including screening for health conditions that may be more common among specific intersex populations.

- **Sexual and Reproductive Health:** Intersex adults may have questions about fertility, sexual function, sexual orientation, and the potential for childbirth. Access to fertility services, counselling, and reproductive options should be provided in a respectful and non-discriminatory manner. Additionally, adults may need support in addressing any physical or psychological impacts of early surgical interventions, such as issues with genital sensitivity or scarring.

- **Mental Health and Social Support:** Many intersex adults face ongoing psychological challenges and may experience gender dysphoria particularly if they underwent non-consensual surgeries or if they feel alienated from societal norms about gender and sex. Mental health support remains crucial, with opportunities for peer connection, therapy, and participation in intersex advocacy networks. For those who experienced trauma related to medical interventions, trauma-informed care is essential.

- **Gender Identity and Expression:** Some intersex individuals may identify with a gender different from the one assigned at birth, or they may prefer to reject traditional gender categories altogether. Adults should have access to gender-affirming healthcare, which includes not only surgical and hormonal options but also support for non-binary or fluid gender expressions.



4.0

Medical Interventions and Ethical Considerations



4.0 Medical Interventions and Ethical Considerations

4.1 History of Medical Interventions on Intersex Individuals

Prior to the 1960s, when intersex children were born, the people around them, who were their parents and doctors, made their best guess and assigned the child a sex. Parents then raised them per social gender norms. Sometimes these intersex children experienced harassment and discrimination as a result of their atypical traits, but many lived well-adjusted lives as adults. During the 1960s, however, and based largely on the unproven recommendations of a single prominent psychologist, John Money, medical norms in advanced countries started changing dramatically. Doctors began recommending surgical solutions to the supposed “problem” of intersex traits.[7]

This history of surgery was also a history of shame and stigmatisation. In some cases, doctors instructed parents to conceal the diagnosis and treatment from the child, instilling feelings of shame in parents and children. Many intersex people did not learn about their variations conditions until they accessed their medical files as adults, sometimes as late as in their 50s, and most of them never knew throughout their lifetime. During the 20th century, medical interventions on intersex individuals were carried out without their consent, as most procedures were performed during infancy or early childhood. The focus was on "fixing" the perceived ambiguity of sex and avoiding future social stigmatisation. These surgeries, often labelled "corrective" or "normalising," were irreversible and typically performed with the understanding that the child would later conform to the assigned gender.

However, numerous intersex individuals grew up experiencing physical and emotional trauma as a result of these surgeries. Many reported significant complications, including loss of sexual sensation, chronic pain, and psychological distress stemming from gender dysphoria or a sense of violation. Some individuals later identified with a gender different from the one assigned at birth, rendering the surgeries not only unnecessary but harmful. These experiences led to growing advocacy against non-consensual medical interventions, framing such practices as violations of bodily autonomy and human rights.[8]

By the late 20th century, intersex individuals and their allies began organising against the medicalisation of intersex bodies. Groups like the Intersex Society of North America (ISNA), founded in 1993, were pivotal in raising awareness about the long-term harm caused by early medical interventions. Intersex activists emphasised the need for self-determination, advocating for a moratorium on non-consensual surgeries and highlighting the importance of bodily integrity.

A landmark moment came in 1996, when intersex activist Cheryl Chase, founder of ISNA, publicly condemned infant genital surgeries at the American Academy of Paediatrics' annual meeting. This event brought intersex issues to the forefront of public consciousness and set the stage for a broader human rights discourse surrounding intersex individuals.

[7] *https://www.hrw.org/report/2017/07/25/i-want-be-nature-made-me/medically-unnecessary-surgeries-intersex-children-us*
[8] *https://chatgpt.com/c/66e6d8f4-2d70-8009-a85a-38a3d9e45e35*

International human rights organisations have increasingly recognised the harmful impact of non-consensual medical interventions on intersex individuals. In 2013, the United Nations condemned these practices, classifying them as human rights violations to torture. The UN Committee Against Torture and the UN Special Rapporteur on Torture have both called for an end to non-consensual surgeries, urging countries to adopt legal frameworks that protect the bodily autonomy of intersex people.

However, despite these changes, the practice of intersex genital mutilation and early surgical intervention remains common in many parts of the world, particularly in Nigeria. Medical protocols continue to vary widely, and in some cases, outdated practices persist. Intersex advocates and human rights organisations continue to push for greater accountability, transparency, and adherence to ethical guidelines in intersex healthcare.

4.2 Ethical Implications of Surgeries and Hormone Treatments

Informed Consent

- Parents of intersex children consent to surgeries based on guidance from medical professionals
- Medical professionals do not always inform parents that their child may not identify with the gender assigned during surgery.
- Based on the rhetoric of medical professionals, surgery is presented as the default plan of action to parents rather than an option.
- Without informed consent, surgical intervention can be recognised as a battery of the child or infant.
- Downplaying alternate outcomes of the surgery raises concerns about whether parents are truly giving informed consent because they do not understand the potential risks of consenting to this procedure.

Principle of Non-maleficence

This is a fundamental ethical principle, particularly prominent in healthcare and bioethics. It essentially means "do no harm" or more completely, the duty to avoid inflicting harm.

- Studies conducted by the 2017 Council of Europe's Committee on Bioethics, regarding intersex surgery have yet to find any quality data confirming its safety and benefits for affected children.[7]
- Adult intersex patients have reported evidence of “genital dysfunction, scarring, loss of sexual feeling, loss of fertility, chronic pain, and the wrong gender assignment—with irreversible excision of genital and gonadal tissues” as a result of genital surgery in their infancy or young childhood. Due to the negligence of their physicians, these patients were subjected to unnecessary and irreversible surgery.
- Acting in the “patient's best interest” is nearly impossible because there is no objective scale to accurately determine each individual patient's best interest as they mature. Physician cannot ensure that a decision as permanent as genital reassignment surgery will have a favorable outcome for an infant or child post-puberty.

- If an intersex infant or child rejects the sex that they were surgically assigned, they may face a lifetime of pain and psychological suffering.

- The performance of medically unnecessary surgery on intersex persons violates the principle of nonmaleficence because the patient is put at high and unreasonable risk of physical harm and the psychological pain associated with rejecting the sex assigned to them.

Principle of Autonomy

- The United States is the only member of the UN that has not ratified the Convention on the Rights of the Child (CRC).

- The CRC endorses the idea that states should "ensure that no one is subjected to unnecessary medical or surgical treatment during infancy or childhood, guarantee bodily integrity, autonomy, and self-determination to children concerned, and provide families with intersex children with adequate counselling and support."

- Dangerous and irreversible intersex surgeries violate a person's right to autonomy over their own future.

- The child should be respected as a member of the decision-making process because even young children, from the age of 6 or 7, can begin to express preferences, understand biological processes, and reflect on their identity.



4.3 Alternative Approaches and the Importance of Patient-Centered Care

Intersex medical intervention alternative approaches focus on delaying non-consensual surgeries, providing informed and holistic care, and centering patient autonomy and decision-making. Patient-centred care is important in these approaches because it prioritises the individual's rights, desires, and long-term well-being over societal norms or cosmetic outcomes. Below are some key alternative approaches:

1

Deferral of Non-Emergent Surgeries

One of the most significant shifts in intersex care is the emphasis on delaying non-emergency medical interventions, such as genital surgeries or hormone treatments, until the individual is old enough to provide fully informed consent. Only interventions that are medically necessary for the immediate health of the child (such as treating life-threatening conditions) are ethical to be performed in infancy. Cosmetic surgeries, particularly those related to gender assignment, are now deferred to allow the individual their autonomy in deciding their own path.

2

Psychosocial and Family Support

Providing psychological counselling and education to families significantly helps them to understand the intersex variation of their child or loved one, which alleviates any societal pressure to intervene early. This support helps families create a positive and accepting environment for their child. Focus on Self-Esteem and Social Inclusion: Intersex children often face societal stigma or exclusion. Counselling and social support help foster resilience and self-acceptance, which is often more beneficial than early medical interventions.

3

Multidisciplinary Care Teams

Setting up holistic and collaborative care is important.
A multidisciplinary team, which should include endocrinologists, urologists, psychologists, ethicists, and intersex human rights organisations, should collaborate to provide comprehensive care. The emphasis is on the physical, emotional, and psychological needs of the individual rather than simply on "normalising" their body.

4

Informed Consent and Patient Autonomy

Central to the alternative approach is ensuring that the individual has the autonomy to make decisions about their body. This means deferring any decisions about surgeries or hormone treatments until the patient can fully understand the risks, benefits, and implications of the procedures. When individuals are old enough to make informed decisions, they should be given clear and unbiased information about their options, including potential long-term outcomes and alternatives to surgery.

5

Gender-affirming and Non-binary Approaches

Affirming Gender Identity: Rather than assuming a binary outcome for sex and gender, alternative approaches to intersex care affirm a wide spectrum of gender identities. This approach avoids coercion into a specific gender role or identity, allowing the individual to explore their own gender without medical pressure.

Non-surgical Gender-Affirming Options: For those who wish to align their physical traits with their gender identity later in life, patient-centred care may include hormone therapy or gender-affirming surgeries based on the individual's informed choice.

Importance of Patient-Centered Care

A patient-centred care model encourages active collaboration and shared decision-making between patients, families, carers, and providers. Patient-centred care is vital in intersex medical interventions because it ensures that the patient's needs, preferences, and rights are the central focus.

Here are some of the things to consider in delivering patient-centred care[9]:

1. Care is collaborative and coordinated and goes beyond physical well-being to also include emotional, social, and financial aspects of a patient's situation.
2. Patients should always be in complete control when it comes to making decisions about their own care and treatment.
3. Clinicians are there to inform, advise, and support, but it is ultimately up to the patient to determine what course of action they will take.
4. Patients need to fully understand the procedures and treatments they will undergo, while clinicians must aim to make them as knowledgeable and comfortable as possible, especially when treating them for a complicated or painful illness or injury.

Approach for patient centered care:

1. Protects autonomy by respecting the individual's autonomy and allowing them to make informed decisions about their bodies and healthcare at the appropriate time in their lives.
2. Reduces Harm: By avoiding premature and non-consensual surgeries, patient-centered care reduces the potential for physical and emotional harm caused by procedures that may not align with the individual's gender identity or desires later in life.
3. Improve the mental health of intersex individuals to feel in control of their bodies and make decisions that are more likely to have positive psychological outcomes, including better self-esteem, reduced anxiety, and less likelihood of trauma associated with early surgeries.
4. Promotes ethical and respectful care that shifts toward patient-centred care aligning with modern bioethical standards, emphasising dignity, respect, and human rights. It avoids the historical harms associated with medicalising intersex bodies to fit societal norms.

[9] *https://ecommons.luc.edu/cgi/viewcontent.cgi?article=1382&context=ures#:~:text=The%20performance% 20of%20medically%20unnecessary,the%20sex%20assigned%20to%20them.*



5.0 Creating Supportive and Inclusive Intersex Healthcare Environment

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5.1 Communication Strategies

Communication is foundational to all layers of the healthcare system but can be problematic. Fortunately, healthcare professionals can take steps to overcome many common barriers to effective communication. Effective communication in intersex healthcare is crucial in creating supportive and inclusive environments. It helps with building trust between the patients and healthcare providers, promoting understanding, and ensuring that intersex individuals and their families feel respected and heard. Healthcare providers must adopt respectful communication practices and employ sensitive strategies for discussing challenging topics and train their staff on the use of intersex-inclusive language.

5.1.1 Principles of Respectful and Effective Communication

Respecting autonomy and individuality:

- **Patient-centred approach**: Respecting the individual's autonomy means placing their needs, desires, and well-being at the centre of care. Healthcare providers must communicate in a way that empowers intersex individuals to make informed decisions about their healthcare. This includes avoiding assumptions based on societal norms or binary notions of sex and gender.

- **Confidentiality and privacy**: It is critical to respect the privacy of intersex individuals. Discussions about their medical conditions, gender identity, or bodily autonomy should be kept confidential unless explicitly consented to by the patient. This ensures trust and protects against unnecessary stigma or exposure.

- **Listening and empathy**: Healthcare providers must prioritise active listening and show empathy when communicating with intersex individuals. Empathy involves understanding the emotional challenges and experiences that intersex people may face without judgement or preconceived notions. This approach fosters a supportive environment where individuals feel valued and understood.

Clarity and Transparency:

- **Using clear, non-jargon language**: Healthcare providers should explain medical information in simple, non-technical language that is accessible to the individual and their family. This promotes better understanding and allows for informed decision-making.

- **Honesty about uncertainties**: If there are uncertainties about the diagnosis or treatment options, healthcare providers should communicate them transparently. It is essential to acknowledge when there are unknowns, particularly with regard to long-term outcomes or non-standardised treatment approaches.

Non-Directive Communication:

- **Avoiding pressuring language**: When discussing healthcare options, particularly surgical or hormonal interventions, it is essential to use non-directive language. Providers should present all options equally, without imposing one course of action as superior or necessary. This gives intersex individuals the space to make decisions based on their values and preferences.

- **Supporting self-expression**: Encourage intersex individuals to express their feelings and concerns freely, especially around topics of gender identity, body image, and healthcare decisions. This open communication promotes self-awareness and empowerment.

5.1.2 Strategies for Discussing Sensitive Topics with Intersex Individuals and their Families

Start with a Foundation of Trust and Empathy

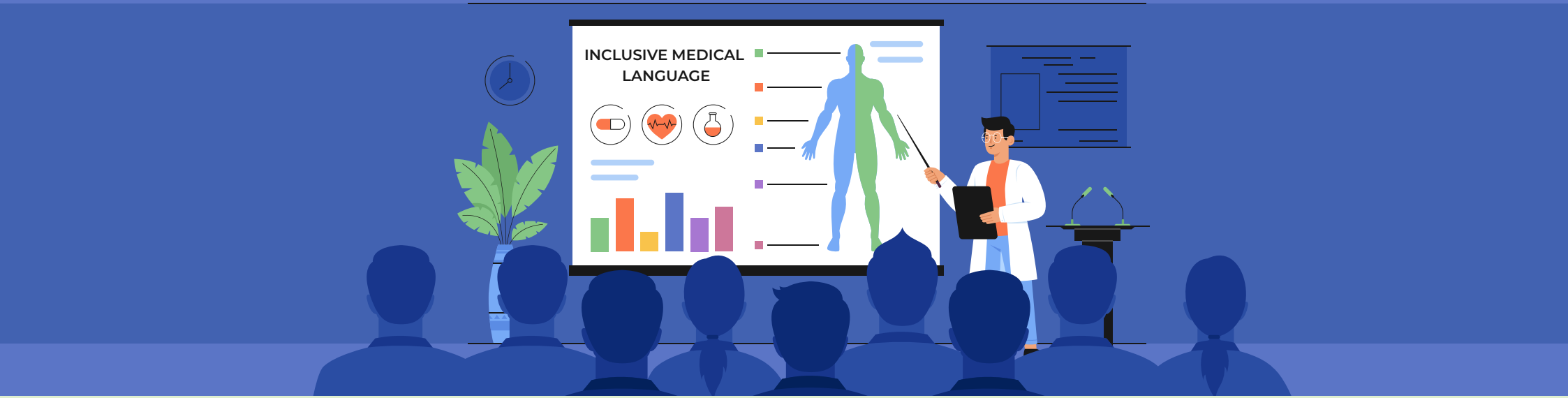
Sensitive conversations about intersex variations, medical interventions, and gender identity require a supportive and non-judgemental environment. Healthcare providers should create an atmosphere of safety by expressing understanding, avoiding rushed conversations, and giving ample time for questions and reflections. It is also important to communicate that intersex variations are natural biological differences, not medical emergencies, and are more common than people think. By normalising intersex conditions, healthcare providers can alleviate the fear and anxiety that often accompany discussions about being "different" from societal norms.

Tailor Communication to the Age and Developmental Stage

For children and adolescents, age-appropriate language is critical when discussing sensitive topics with an intersex youth. Healthcare providers should avoid overwhelming young people with complex medical information while ensuring they understand the key aspects of their condition. Encourage curiosity and exploration of gender identity in a supportive manner. Parents often feel anxiety or guilt when learning about their child's intersex variation. Address these emotions with compassion and avoid framing the conversation as one where there is something "wrong" with the child. Instead, emphasise that the healthcare team's role is to support the child's well-being, and that's exactly what you are going to do.

Discussing Medical Interventions

When discussing medical interventions, especially non-emergency surgeries or hormone treatments, ensure that the conversation is non-directive and unbiased. Present the risks and benefits of each option, including emphasising the option to delay any interventions until the individual is capable of making their own decisions. This approach helps the family feel in control while promoting the long-term well-being of the intersex individual. Encourage families to take their time in making decisions about irreversible procedures. Explain that delaying certain medical interventions, such as gender-affirming surgeries, allows the individual to be actively involved in the decision-making process later in life.



5.1.3 Training Staff on Intersex Language and Communication

Language is inclusive when we use words in ways that demonstrate our respect for how people describe their own bodies, genders, and relationships. It is important for everyone to show this respect even when we are describing in their absence, and it is particularly essential for healthcare providers to ensure a supportive, friendly, and inclusive healthcare practice.

Researchers have found that how we are described by others has an enormous impact on our health and wellbeing, as well as on how likely we are to seek healthcare when necessary.[10]

- **Avoid Stigmatising Language**: Healthcare staff should avoid terms that pathologise or stigmatise intersex variations. Words like “disorder,” “abnormal,” or “ambiguous” can be harmful and reinforce negative stereotypes. Instead, use respectful terms such as "intersex variation" or "differences in sex development (DSD)," and always follow the patient's preferences for language, as some might prefer none of the above.

- **Cultural Sensitivity Training**: Staff should receive training on cultural competence, particularly with our context here in Nigeria, where there's diverse cultural and religious attitudes toward sex, gender, and identity. This training will help health service providers navigate sensitive topics with awareness of cultural dynamics while still adhering to ethical and human rights principles.

- **Education on Intersex Rights**: Educating healthcare staff on the rights of intersex individuals, including the right to informed consent, bodily autonomy, and access to non-discriminatory healthcare, is very important. Staff should understand that early, non-consensual surgeries have evolved and are no longer considered best practice, and the emphasis should be on the individual's long-term mental and physical well-being.

[10] *https://www.wolterskluwer.com/en/expert-insights/why-patientcentered-care-is-so-important*

- **Training in trauma-informed practices:** Many intersex individuals, especially those who have undergone non-consensual medical interventions, often experience trauma-related to their healthcare experiences. Train staff to recognise signs of trauma and respond with sensitivity, ensuring that care is provided in a manner that avoids re-traumatisation and promotes healing and overall wellbeing.

- **Encouraging emotional support**: Staff should be trained to offer emotional support and refer individuals to counselling services or support groups when needed. Understanding the psychological impact of being intersex, particularly in environments where stigma is prevalent, can help staff provide compassionate and effective care.

To-Go List for Intersex Patient Communication

A critical aspect of affirming care is to use inclusive language when communicating with all patients:

1. Document the patient's information in electronic health records
2. Use the person's chosen name and pronouns consistently across the health center
3. Recheck frequently, as this information may change
4. Mirror the terms patients use to describe themselves, their body parts, and their diagnoses
5. Use gender-inclusive language and avoid gendered terms. For example, instead of asking, "Do you have a boyfriend?" Ask, "Are you in a relationship?" instead of requesting "Please remove your bra and pants," say "Please remove your underwears"
6. Only ask questions relevant to the patient's current health needs. Ask yourself: "What do I know? What more do I need to know to treat this patient? How can I ask for the information I need to know in a sensitive way."
7. Ensure that communication is two-way, authentic, and active
8. Examine one's own implicit and explicit biases about gender identity, gender expression, and anatomy.
9. Participate in training sessions on Intersex+ affirming health care to ensure you are using current communication strategies and following up-to-date protocols for intersex care.

5.1.4 Best Practices for Obtaining and Documenting Informed Consent.

In order to ensure that the patient has been given sufficient information with which to make an informed decision as to the course of his/her medical treatment, the following should generally be discussed and documented in the medical record[11]:

1. Details of the nature of the patient's illness and diagnosis
2. Indications for the proposed treatment plan, procedure, or medication, as well as the anticipated prognosis
3. A description of the proposed treatment or procedure, including medication that will be prescribed, and its purpose
4. The probable outcome, particularly if it is difficult to predict, and the patient's expected post-procedure/treatment course
5. Potential modifications or extensions of the treatment or procedure
6. The possible benefits of having the procedure
7. The most likely and severe risks and side effects of the procedure and treatment or medication, preceded by a general inclusive statement, such as "including but not limited to..."
8. A statement about unexpected complications/additional medical procedures
9. Reasonable alternative methods of treatment or no treatment, including the risks, benefits, and prognosis associated with each alternative or with no treatment
10. Notification of experimental or investigatory procedures

[11] *https://www.parliament.nsw.gov.au/lcdocs/other/12075/Inclusive%20language%20guide%20-%20Respecting %20people%20of%20intersex,%20trans%20and%20gender%20diverse%20experience.pdf*

Guidelines for Informed Consent Medical Interventions

Informed consent in medical interventions that are focused on "normalising" the bodies of intersex individuals is a critical consideration that centres on the rights of the intersex patient to make autonomous decisions regarding their own health, just as mentioned above.

Health care practitioners should:

1. Allow intersex individuals their rights to make informed decisions about their own bodies. Medical intervention should respect the bodily autonomy and integrity of the patient.

2. Refrain from giving non-emergency medical interventions to intersex infants and teenagers, especially those that are irreversible; they should be postponed until the individual is old enough to provide informed consent.

3. Give their patients the information they ask for or need about their intersex variation, its treatment, and their prognosis.

4. Ensure that if medical interventions are deemed necessary for the health of the infant or child, they must be limited to lifesaving or urgent medical needs.

5. Inform their patients in the way they can best understand it. The information must be given in a language that the patient understands and in a manner that takes into account the patient's level of literacy, understanding, values, and belief systems.

6. Ensure that when minors are involved, decisions should be made in consultation with them in an age-appropriate manner, involving a multidisciplinary team that includes psychologists and medical ethicists.

7. Make it a priority to provide comprehensive, transparent, and easily understandable information about the medical options available, including:
    1. The nature and purpose of the proposed intervention
    2. Alternatives to the proposed intervention, including the option of not undergoing any intervention.
    3. The potential risks, benefits, and long-term outcomes of each option.

8. Information should include the possibility of physical implications, psychological effects, and the impact on fertility and sexual function.

9. Refrain from withholding any information, investigation, treatment, or procedure the health care practitioner knows would be in the patient's best interest.

10. Apply the principle of informed consent as an on-going process

11. Allow patients access to their medical records[12]

12. Healthcare professionals should be aware of and comply with international human rights standards, which call for the protection of intersex individuals from unnecessary and irreversible medical interventions.

13. The process of obtaining informed consent should be well documented, with clear records of all discussions and decisions made by the patient or parents/guardians.

Consent should be revisited periodically, especially in cases of long-term or staged medical interventions. Patients should be able to withdraw consent at any time if they reconsider their decisions.



[12] *https://www.svmic.com/resources/newsletters/218/informed-consent-best-practices*

5.2 Training For Healthcare Professionals

To design an effective training program for healthcare professionals, you will need to implement a multi-faceted approach that combines accurate information, real-life case studies, and interactive learning activities.

Importance of a Structured Medical Curriculum on Intersex issues

1. It bridges critical knowledge gaps: Intersex-specific education helps future healthcare providers understand the biological, psychological, and social aspects of intersex variations, correcting long-standing misconceptions and medical myths.

2. It encourages ethical and right-based: Training on informed consent and bodily autonomy ensures healthcare professionals respect the rights of intersex individuals, especially in avoiding non-consensual, irreversible interventions on minors

3. It improves communication and promotes inclusive language usage in healthcare centers: Teaching inclusive, non-stigmatising terminology helps reduce bias and discrimination, making intersex patients feel safe, seen, and respected.

4. Training equips healthcare providers to recognise and respond to the emotional impacts of stigma and medical trauma, integrating psychological support into patient care.

5. Training enables healthcare workers to provide care that is both sensitive to local values and grounded in human rights especially in a cultural and religious context.

6. It encourages interdisciplinary collaboration in intersex healthcare provision including the service of psychologists, intersex human rights organisations, urologist, counsellors etc. This promotes a holistic care approach in addressing all dimensions of intersex health.

7. Education and training helps healthcare workers handle sensitive decisions involving intersex infants and adolescents which will reinforce critical thinking and accountability.

5.3 Confidentiality and Privacy

Robust patient privacy and confidentiality are a fundamental part of the healthcare system. Protecting the intimate information entrusted to you as medical professionals is part and parcel of your job and responsibility.

But in a world that is becoming ever more digital, how can healthcare providers be sure that they are keeping patient information appropriately protected at all times? This is a growing concern amongst intersex patients and their families. In the digital age, we also have more need than ever to connect different parts of the

healthcare system to create a seamless experience for both doctors and patients.

So, how can we keep information safe in the modern medical environment?

Ensuring Confidentiality in Patient Records and Communications.

1. Create thorough policies and confidentiality agreements
Drawing up all-encompassing and wide-ranging confidentiality agreements or policies means that everybody on your medical team knows exactly what is expected of them in every eventuality.

2. Provide regular training
Holding regular training sessions for all your staff members, from administrators to doctors and nurses, helps to reinforce how essential confidentiality requirements are and reminds staff of their duties and expectations.

3. Make sure all information is stored on secure systems
It is essential that the highest level of security and digital protection is used when storing patient data. Purchasing platforms or using cloud providers that ensure your data is safe is the best way to look after this.

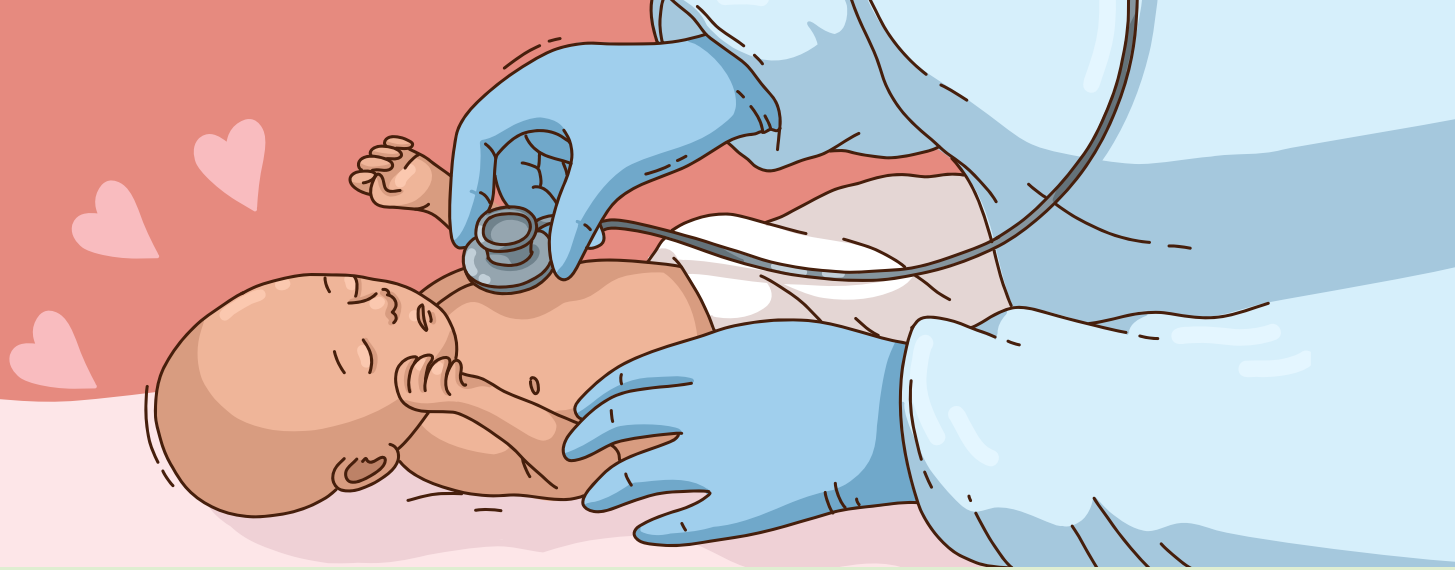
Furthermore, it is important that only strictly necessary personnel have access to this data. Levels of password protection that control access are also worth considering and investing in.

4. No mobile phones
An easy way to eliminate possible threats to patient confidentiality is to strictly limit or remove mobile phones from patient areas. This ensures that no one can either maliciously or accidentally record or photograph private records or information. According to research by Imperial College Healthcare NHS Trust in London, 65% of doctors used SMS to communicate with colleagues about a patient, opening up concerns about privacy.[13]

This can sometimes be a difficult rule to enforce given the proliferation of digital devices. However, regularly reminding staff and patients why it is in their best interests can help to reduce any resistance.

5. Think about printing
Once all your technical solutions and security are in place, it can be tempting to think you have everything sorted. However, printed materials that contain key patient information are often overlooked. Labels, forms, and printed notes can easily be misplaced or even stolen if they are in a busy area. Having streamlined, easy-to-use and secure printing systems is well worth investing in.

[13] *https://www.svmic.com/resources/newsletters/218/informed-consent-best-practices*



5.4 LEGAL AND ETHICAL OBLIGATIONS FOR PROTECTING PATIENT PRIVACY

Trust is a fundamental element of the healthcare provider-patient relationship, and building trust helps ensure that patients disclose accurate and thorough health information to doctors and other healthcare personnel. In turn, healthcare providers and staff members have an ethical and legal responsibility to protect patient confidentiality and prevent unauthorised disclosure of patients' protected health information (PHI)[14].

Both state and federal laws address patient confidentiality and the release of health information. Some state laws are more specific than federal laws about certain aspects of confidentiality, although there's none specific for intersex people, but the Child Rights Act (CRA) covers for children. Healthcare Providers and staff members should understand these legal requirements and their role in upholding existing laws.

[14] *https://www.samedical.org/images/attachments/guideline-on-informed-consent-jul012.pdf*

Acting Within the Law

1. Doctors, like everyone else, must comply with the law when using, accessing, or disclosing personal information. The law governing the use and disclosure of personal information is complex and varies across Nigeria.

    - **Medical and Dental Practitioners Act**: This act governs the practice of medicine and dentistry in Nigeria. It requires healthcare providers to maintain patient confidentiality and only disclose information with the patient's consent, except in cases where the law requires mandatory reporting (e.g., in public health emergencies). Intersex patients are entitled to the same protections under this act.

    - **Child Rights Act (For Minors)**: For intersex minors, the Nigerian Child Rights Act outlines protections for the child's privacy, which includes medical information. However, parents or guardians

are often involved in the decision-making process for minors. It is critical that healthcare providers ensure that privacy is still respected, particularly in terms of when and how sensitive information is shared.

- **Nigerian Data Protection Regulation (NDPR)**: The NDPR, implemented by the National Information Technology Development Agency (NITDA), establishes guidelines for protecting personal data in Nigeria. While it is not specific to healthcare, it applies to all personal information, including medical data. This regulation mandates that individuals' data, including health records, must be processed and stored securely and only used for the purposes for which it was collected.

- **Patient Confidentiality Laws**: Although there is no explicit law focused on intersex individuals, general patient confidentiality laws in Nigeria require healthcare providers to keep all medical records confidential. These laws prevent the unauthorised disclosure of patient information to third parties without the explicit consent of the patient.

- **Data Protection in Healthcare Systems**: Under the NDPR, healthcare facilities must implement strong security measures to protect patient data from unauthorised access. This includes using secure electronic medical record systems and ensuring that only authorised personnel can access sensitive medical information.

- **Code of Medical Ethics in Nigeria**: The Medical and Dental Council of Nigeria (MDCN) provides guidelines that outline the duty of healthcare providers to maintain patient confidentiality. Violations of these guidelines can lead to disciplinary action, including suspension or revocation of a medical license.

- **Patient Rights Under Nigerian Law**: Nigerian law upholds the right of patients, including intersex individuals, to make decisions about their own bodies and healthcare. These rights are protected under general health and privacy laws, requiring healthcare professionals to ensure that individuals' medical choices are respected and protected from public scrutiny.

- **Legal Guardianship and Privacy for Minors**: For minors, the Child Rights Act provides that parents or guardians have a legal responsibility to make decisions that are in the best interest of the child. However, the healthcare provider must ensure that decisions are made in a manner that respects the privacy and future autonomy of the child.

- **Redress for Breaches of Privacy**: Under Nigerian law, intersex individuals whose privacy has been breached may have legal recourse. Complaints can be filed with the Nigerian Medical and Dental Council for professional misconduct or under the NDPR for data privacy violations. Healthcare providers found guilty of violating privacy obligations may face legal consequences, including fines or the loss of professional licenses.

2. If you are not sure how the law applies in a particular situation, you should consult a data guardian, a data protection officer, your defence body or professional association, or seek independent legal advice.

Why Inclusive Policies and Standard Operating Procedure for Intersex Healthcare in Nigeria are Critical

1. **Human Rights Protection:** Inclusive policies rooted in human rights frameworks are essential for safeguarding the bodily autonomy of intersex individuals in Nigeria as it ensures that no medical intervention is carried out without full, informed, and personal consent that aligns national practice with global standards like the Yogyakarta Principles[15].

2. **Prohibits Discrimination and ensures Equal Access:** Explicitly including anti-discrimination clauses in healthcare laws and training professionals to treat all individuals with respect and dignity ensures that intersex individuals are not denied access to care due to their sex characteristics.

3. **Addressing Psychological Harm through Trauma-Informed Care:** Inclusive healthcare policies adopts trauma-informed care practices to provide healing-centered, non-retraumatizing environments.

4. **Advancing Legal Recognition and Gender Autonomy:** Inclusive healthcare policies is backed by legal frameworks that allow intersex individuals to self-identify and access legal recognition without undergoing invasive procedures. Creating supportive legal pathways ensures that healthcare systems respect and affirm gender diversity, reinforcing dignity and freedom of identity.

5. **Facilitates Access to Justice and Redress:** Where rights are violated especially through non-consensual surgeries or denial of care, an inlusive policy will set modalities for intersex individuals' access to justice. Healthcare policies must include mechanisms for legal redress, compensation, and accountability. Such provisions strengthen public trust in the health system and affirm that violations against intersex individuals will not be tolerated or ignored under Nigerian law.

[15] *https://www.hospitalhealth.com.au/content/facility-admin/sponsored/5-important-ways-to-maintain-patient-confidentiality-1425215591*

6.0

Cultural Competence and Sensitivity



6.0 Cultural Competence and Sensitivity

6.1 Understanding Cultural Diversity and its Impact on Intersex Healthcare

Nigeria has a vast cultural diversity, with over 250 ethnic groups and a wide range of religious practices, with significantly shaped societal views on gender, identity, and health. This diversity is crucial to understanding the healthcare challenges and barriers faced by intersex individuals in the country. In a society where traditional gender norms are deeply rooted in cultural and religious beliefs, intersex individuals whose physical traits do not fit into the conventional definitions of male or female often face social stigma and marginalisation. For healthcare providers, navigating this cultural landscape requires sensitivity, respect for local traditions, and a commitment to patient-centred care that protects the dignity and rights of intersex people.

Cultural diversity significantly shapes intersex healthcare in Nigeria, especially through rigid gender norms, religious beliefs, and traditional practices. Intersex individuals often face intense pressure from families and communities to conform to binary gender roles through early medical interventions. Religion, especially Christianity and Islam, reinforces these binary views and influences family decisions, often contradicting modern medical and human rights standards. Traditional beliefs tied to roles like marriage and fertility further push for unnecessary medical procedures, particularly in rural areas.

Healthcare professionals may also carry cultural or religious biases, leading to discrimination, insensitive treatment, or denial of care. To address these issues, healthcare workers need proper training on intersex healthcare, respectful communication, and cultural sensitivity. Building trust with intersex patients and their families requires empathy, clear information, and a commitment to prioritising the individual's rights, health, and autonomy over societal expectations.

Strategies for Healthcare workers in Providing Culturally Sensitive Intersex Care.

Cultural competency is necessary to provide safe care that eliminates mistrust and closes the gap on healthcare disparities.

- Engage in critical reflection of your own values, attitudes, and beliefs.
- Seek learning opportunities related to care for intersex communities and people.
- Become an intersex ally.
- Ask your intersex patients how they prefer to be addressed (by which name preferred).
- Do not make assumptions based on appearance and/or dressing.
- Be careful of language that labels.
- Avoid assumptions regarding someone's gender identity and sexual preferences.
- Identify yourself as an intersex-friendly healthcare provider and make proof with knowledge on intersex rights.
- Create a welcoming environment by posting an intersex-inclusive nondiscrimination and patient bill of rights around your healthcare facility or centre.
- Collect feedback from intersex patients and their families.

- Modify forms to include gender-neutral language.
- Include cultural competency continuing education for all staff annually.
- Avoid gender-based assumptions when selecting hospital clothing for intersex patients.
- Display intersex-friendly symbols or signs where patients can see them.
- Consider issues of personal privacy related to nurse-patient conversations.
- Consider issues of personal privacy when the patient is in an open holding area and/or intake area.
- Provide patients and family/friends a copy of the policy for handling intersex-based grievances.

Addressing Biases And Fostering An Inclusive Healthcare Environment.

One of the most pressing challenges we have witnessed is the bias that exists within healthcare systems, particularly towards intersex people. In Nigeria, where cultural, religious, and societal norms often reinforce binary views of sex and gender, healthcare providers play a vital role in either perpetuating or challenging these norms. Unfortunately, many healthcare providers have not fully understood the unique needs and experiences of intersex individuals, leading to biased treatment, misinformed medical decisions, and a lack of inclusive care. Addressing these biases and fostering an inclusive healthcare environment requires deliberate actions, including education, policy changes, and a commitment to patient-centred care.

1. Education and Training on Intersex Issues
An important step in addressing biases with healthcare systems and facilities in Nigeria is to provide comprehensive education and training on intersex issues. Healthcare providers must be educated about the natural diversity of intersex traits. Training should focus on ending harmful, non-consensual medical practices and promoting ethical, compassionate care.

2. Emphasising the Importance of Informed Consent
Informed consent is a critical principle in all areas of healthcare, but it is particularly important in the treatment of intersex individuals. Historically, many intersex people in Nigeria and globally have been subjected to medical interventions without their informed consent, often in infancy or early childhood. This practice not only violates the autonomy of the individual but also contributes to a lifetime of psychological and physical trauma.
Healthcare providers must be trained to respect the rights of intersex individuals to make informed decisions about their own bodies.

3. Creating a Culturally Competent and Sensitive Approach
Healthcare providers must be trained to engage in culturally sensitive conversations with intersex individuals and their families. For instance, when discussing a child's intersex traits, healthcare providers should avoid stigmatising language and instead offer reassurance that intersex variations are a natural part of human diversity. They should also be prepared to address cultural or religious concerns with empathy, helping families navigate these complex issues while keeping the well-being of the intersex individual at the forefront.

4. Using Inclusive and Affirming Language
One of the most effective ways healthcare providers can address bias is by adopting inclusive and affirming language when interacting with intersex individuals. Language plays a powerful role in shaping attitudes and perceptions and using disrespectful or

stigmatising terms can reinforce harmful biases and make intersex patients feel alienated or disrespected.
Healthcare providers and every staff member. This simple act of respect can go a long way in making intersex patients feel valued, seen, heard and supported.

5. Implementing Policies to Protect Intersex Rights
Hospitals, clinics, and public health centres must adopt policies that explicitly protect the rights of intersex individuals. Anti-discrimination policies that protect intersex individuals from biased treatment, harassment, or denial of care based on their sex characteristics should also be developed and institutionalised.

6. Providing Mental Health Support
Intersex individuals often face significant psychological challenges related to their experiences of stigma, discrimination, and medicalisation. To create a truly inclusive healthcare environment, providers must offer comprehensive mental health support for intersex patients, recognising the emotional and psychological impact of growing up in a society that does not fully accept their differences.

7.0 Case Studies and Best Practices



7.0 Case Studies and Best Practice



7.1 Case Studies

Real-World Examples of Intersex-Friendly Healthcare Practices.

Human Rights Watch's advocacy[16] efforts and several U.S. hospitals are developing new policies to train medical teams on intersex issues and protect patients' rights to autonomy and informed consent. In some cases, hospitals have also created intersex-friendly intake forms that recognise non-binary or diverse gender identities, which helps foster a more inclusive and respectful healthcare environment. In Australia, Intersex Human Rights Australia[17] and Intersex Peer Support Australia have developed community-based programs where intersex individuals can connect with one another and receive peer support alongside their medical care. These networks provide a safe space for intersex people to discuss their experiences and access affirming care without fear of discrimination. Peer support has been shown to reduce the emotional burden of feeling isolated or misunderstood by medical professionals.

Countries such as Germany and Chile have passed legislation to postpone non-consensual surgeries on intersex infants. This change came after years of advocacy from intersex rights organisations and research showing the harm caused by early surgeries performed to "normalise" genital appearance. These laws allow intersex individuals the time to grow and decide for themselves if they wish to undergo medical interventions, promoting a more ethical approach to intersex healthcare.[18]

Healthcare providers such as those at Boston Children's Hospital and the University of Michigan have adopted multidisciplinary models for intersex care[19]. These teams consist of endocrinologists, urologists, psychologists, social workers, and ethicists who work together to ensure that intersex patients receive comprehensive care that addresses both their physical and emotional needs. These institutions emphasise the importance of shared decision-making, where patients and their families are provided with all relevant information before making healthcare decisions.

Howard Brown Health in Chicago integrates mental health services into its care model for intersex patients[20]. Recognising the psychological impacts of stigma and medical trauma, Howard Brown offers counselling and peer support for intersex individuals and their families. This holistic approach ensures that healthcare addresses not just the medical but also the emotional and social challenges that come with being intersex.

[16] https://www.medpro.com/protecting-patient-confidentiality#:~:text=In%20turn%2C%20healthcare%20providers%20and,and%20release%20of%20health%20information.

7.2 Best Practices - Success Stories of Countries Advocating for the Health and Rights of Intersex Persons.



South Africa

In South Africa, in terms of section 2 of the Alteration of Sex Description Act of 2003, intersex persons can alter their name and sex on birth certificates to suit preferred gender roles. The Promotion of Equality and Prevention of Unfair Discrimination Act of 2004 interprets the definition of 'sex' to include intersex persons; therefore, intersex persons are equally protected from discrimination on the basis of sex.[21]



Kenya

Section 10(3) of the Persons Deprived of Liberty Act of 2014 in Kenya grants intersex persons deprived of liberty the right to determine their gender when conducting a body search on them recognizing bodily autonomy in a context where individuals are most vulnerable[22]. In 2019 Kenya made history as the first African country to enumerate intersex individuals in its national census by introducing an "I" sex marker—a milestone affirming legal recognition and enabling inclusion in national planning[23]. Civil society has been instrumental in this progress: the Intersex Persons Society of Kenya (IPSK), Jinsiangu, and other organizations have driven awareness campaigns, policy engagement, and community support, while KNCHR has actively championed reform, including through public consultations and drafting the Intersex Persons Bill.

[17] *https://en.wikipedia.org/wiki/Yogyakarta_Principles*
[18] *https://ihra.org.au/inclusion/*
[19] *Intersex Human Rights Australia*
[20] *https://www.hrw.org/report/2017/10/26/changing-paradigm/us-medical-provider-discomfort-intersex-care-practices*
[21] *https://www.hrw.org/report/2017/10/26/changing-paradigm/us-medical-provider-discomfort-intersex-care-practices*
[22] *https://howardbrown.org/supporting-intersex-identities-in-healthcare/*
[23] *https://www.chr.up.ac.za/the-intersex-project*

Malta



Malta became the first country to ban non-consensual medical interventions on intersex children in 2015, establishing a precedent for legal protection with the Gender Identity, Gender Expression, and Sex Characteristics Act (2015). This does not only prohibits non-consensual medical interventions on intersex individuals but also provides legal avenues for individuals to seek redress if their rights are violated. This includes the right to compensation for physical or psychological harm resulting from such interventions.[24]



Other International frameworks: In addition to the above some international frameworks have recognised and demanded the protection of intersex people. Some of this frameworks include the United Nations Human Rights Committee Resolution 55/14 (adopted in April 2024)[25] which calls on all states to combat discrimination, violence and harmful practices against intersex persons; and Africa Commission on Human and People's Rights (ACHPR) resolution 552 (adopted in March 2023)[26] which urges the promotion and protection of the rights of intersex persons across Africa.

Iceland



Intersex Ísland, founded in 2014 to provide a safe space for intersex individuals, to educate the public about intersex issues, and to advocate for intersex rights- worked with a team of linguistic experts to develop a language base of non-pathologising intersex terminology. Through public education and political advocacy, Intersex Ísland has been shaping an alternative discourse around intersex in the Icelandic language. In a remarkably short time, the organisation has been able to engage Iceland's political parties, parliamentarians, and key government ministers, some of whom have spoken out publicly against non-consensual surgeries on intersex children. Because of Intersex Ísland's efforts, intersex information is now included in educational materials that go to all schools in the Reykjavik area and are accessible to all schools in the country[27].

[24] *https://klrc.go.ke/index.php/projects/on-going-projects/612-taskforce-on-policy-legal-institutional-and-administrative-reforms-regarding-intersex-persons-in-kenya*
[25] *https://rightsafrica.com/2022/10/28/kenya-passes-landmark-law-to-recognize-and-protect-intersex-people/*
[26] *https://www.consilium.europa.eu/en/press/press-releases/2017/02/03/malta-declaration/*
[27] *https://docs.un.org/en/A/HRC/RES/55/14*

Australia



Australia is recognized as a leader in intersex health rights, combining legal protections, policy reform, and peer-led advocacy. The Sex Discrimination Amendment Act 2013 made “intersex status” a protected characteristic, banning discrimination in employment, education, and services[28] . Following recommendations from the Australian Human Rights Commission (2021), the Australian Capital Territory passed the Variation in Sex Characteristics (Restricted Medical Treatment) Act 2023, the first law in Australia to ban non-consensual medical interventions on intersex minors, ensure oversight, and provide psychosocial support[29].

Greece



Greece has made significant strides in advancing intersex health and human rights through landmark legislation and inclusive policymaking. In a historic vote on July 19, 2022, the Greek Parliament passed the Medically Assisted Reproduction Reforms Act (Law 4958/2022), banning non-consensual surgeries and hormonal interventions on intersex minors under age 15 unless absolutely necessary, court-approved, and backed by expert interdisciplinary opinion—enforcing serious sanctions for violations, including fines, imprisonment, and professional bans for medical practitioners[30] . Additionally, Greece’s 2023 Law 5029 on violence and bullying protection includes "sex characteristics" as a protected ground in school environments, reinforcing legal protections against intersex discrimination[31]

[28] *https://achpr.au.int/en/adopted-resolutions/resolution-promotion-and-protection-rights-intersex-persons/*
[29] *https://www.arcusfoundation.org/wp-content/uploads/2016/08/AstraeaWeAreRealReport2.pdf*
[30] *https://www5.austlii.edu.au/au/legis/cth/bill_em/sdaogiaisb2013865/memo_0.html*
[31] *https://humanrights.gov.au/about/news/media-releases/commission-welcomes-passage-new-act-intersex-laws*

8.0 Call to Action



8.0 Call to Action

1. Delaying Early Surgical Interventions

Research increasingly supports delaying medically unnecessary surgeries on intersex infants and children until they can participate in decision-making. Studies show that early genital surgeries can lead to physical and psychological harm, especially when they do not align with the individual's gender identity later in life.
This trend aligns with recommendations from advocacy groups like Human Rights Watch and interACT, which promote patient autonomy.

2. Focus on Hormonal Health and Long-term Monitoring

Emerging research highlights the importance of hormonal management for intersex individuals, especially those who have undergone gonadectomy (removal of gonads). For example, individuals with conditions like complete androgen insensitivity syndrome (CAIS) may require lifelong hormone replacement therapy to maintain bone health and manage other secondary effects of their intersex variation.
Research also emphasises the need for better long-term care guidelines to address issues like metabolic health, fertility, and cancer risks associated with some intersex variations.

3. Mental Health and Psychosocial Support

Mental health research is gaining traction, with studies focusing on the social and emotional impacts of intersex variations. Many intersex individuals experience stigma, social isolation, or trauma related to medical interventions. Institutions like Howard Brown Health are integrating mental health services into their intersex healthcare offerings, recognising that holistic care, including access to support groups and counselling, is crucial.

4. Patient-Centered Care Models

Research is also exploring the benefits of multidisciplinary care models for intersex patients, involving endocrinologists, urologists, mental health providers, and social workers. This approach aims to provide comprehensive care that goes beyond immediate physical health needs. In countries like Australia, organisations like Intersex Human Rights Australia are leading the charge in developing inclusive care frameworks that prioritise patient autonomy and informed consent.

5. Data Collection and Research Gaps

Another trend is the call for better data collection on intersex populations. Researchers and advocates emphasise the need for improved medical records, standardised data collection on health outcomes, and inclusive intake forms that reflect the diversity of intersex experiences[32].
These efforts are aimed at closing research gaps and improving evidence-based care, especially in areas like fertility counselling, cancer prevention, and long-term health monitoring.

6. Cancer and Fertility Research

New research is delving into the increased cancer risks associated with some intersex variations, particularly those involving atypical genders. For example, individuals with Swyer syndrome or other DSD variations may have a higher risk of developing germ cell tumours if their gonads are not removed. Fertility options for intersex individuals, such as testicular sperm extraction or in vitro fertilisation (IVF), are also becoming a growing area of clinical interest, though there remain significant knowledge gaps.

https://en.wikipedia.org/wiki/Intersex_healthcare

7. Policy Shifts and Advocacy

At the policy level, organisations like the American Medical Association and the American Academy of Paediatrics are reviewing their guidelines in response to pressure from advocacy groups. These groups are urging a moratorium on surgeries that are not medically necessary until the patient can meaningfully participate in decisions. HRW

This shift reflects a broader trend toward aligning medical practices with human rights frameworks.

[32] *https://www.oiieurope.org/greek-parliament-votes-to-ban-igm/*
[33] *https://www.oiieurope.org/good-practice-map-2023/*
[34] *https://ihra.org.au/inclusion/*

Conclusion



Summary of the Toolkit's Content

This Intersex-Friendly Healthcare Toolkit is designed as a comprehensive resource aimed at healthcare providers, policymakers, and community leaders to improve healthcare delivery for intersex individuals. The toolkit serves several key purposes:

1. **Education and Awareness**: It aims to raise awareness among healthcare professionals about intersex variations and dispel outdated medical practices that prioritise body normalisation over patient autonomy. The toolkit emphasises the need for healthcare systems to shift towards recognising bodily diversity as natural and avoiding unnecessary medical interventions.

2. **Ethical Medical Interventions**: The toolkit promotes patient-centred care, with a strong focus on informed consent, bodily autonomy, and delaying non-urgent medical interventions, especially non-emergency surgeries on intersex infants.

3. **Cultural Competence**: Understanding the cultural context, especially in regions like Nigeria, is essential. The toolkit provides strategies for engaging with patients and their families in a respectful manner that aligns with both cultural values and human rights.

4. **Advocacy and empowerment**: It equips intersex individuals and their families with the tools to advocate for their rights within healthcare settings. The focus is on promoting self-advocacy, mental health support, and creating environments that are respectful of intersex identities.

5. **Policy Development**: The toolkit also emphasised the importance and need for an inclusive and comprehensive policy framework for intersex healthcare.

6. **Informed Consent and Ethical Practice**: There is a significant emphasis on upholding ethical standards, particularly in relation to informed consent, and ensuring medical decisions are made with the full involvement of the patient or their guardians. The toolkit stresses that any medical interventions must respect the bodily autonomy of intersex individuals.

7. **Training and Support**: The toolkit calls for ongoing training programs for healthcare providers, enhancing their understanding of intersex issues and preparing them to provide non-discriminatory care. It also encourages institutional support to create a healthcare environment that is inclusive of intersex people.

8. **Resources and Tools**: Finally, the toolkit offers resources, including case studies, best practices, and templates, to aid healthcare providers in implementing intersex inclusive policies and practices.



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